HCAI 2023 DA Executive Presentation

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C699 Cohort

CalPERS & Multi-County California Employers

Inpatient Hospital Pricing Disparities & Fiduciary Cost Containment



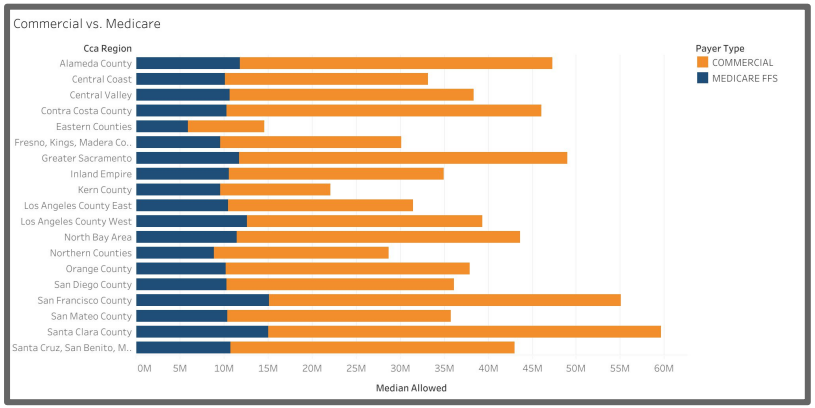
- **Primary Stakeholder Focus:**
  - Public purchasers like CalPERS covering over 1.5 million state and municipal lives across California.
  - Multi-county commercial employers exposed to varying county-level hospital pricing structure
- **Fiduciary Mandate:**
  - Enforcing Employee Benefits Security Administration (EBSA) and Consolidated Appropriations Act requirements to audit plan costs and prevent overpayments.
- **Core Research Objective:**
  - Evaluate California Department of Health Care Access and Information (HCAI) 2023 inpatient claims data to verify whether pricing spreads stem from patient health needs or regional hospital provider leverage.



Illustrations:Statewide Inpatient Pricing: Commercial vs. Medicare

Private Plans Pay Double the Federal Baseline Across California

- **1.97x Statewide Multiplier:** Commercial inpatient payments double federal rates for comparable diagnostic codes.
- **Commercial Allowed Median: $25,200** per hospital stay statewide.
- **Medicare Baseline: $17,201** per hospital stay statewide.
- **Systemic Pressure:** Georgetown University CHIR findings indicate unmanaged hospital payment growth strains private health funds alongside federal trust solvency.



Insights: Regional Variance & Market Concentration

Hospital Provider Leverage Dictates Local Employer Costs

- **Competitive Baseline (Kern County):**
  - Commercial pricing aligns near federal baselines at **1.27x Medicare** ($14,809 average).
- **Consolidated Surcharge (Central Coast):**
  - Santa Cruz, San Benito, and Monterey counties average a **2.66x multiple**, reaching up to **3.15x Medicare**.
  - Reflects RAND Corporation findings on hospital network consolidation driving higher negotiated prices.
- **Intra-County Disparity (Los Angeles County):**
  - West Los Angeles (**1.73x**) vs. East Los Angeles (**1.45x**).
  - Represents an excess difference of **$4,900 - $6,300 per admission** inside the same metropolitan area.

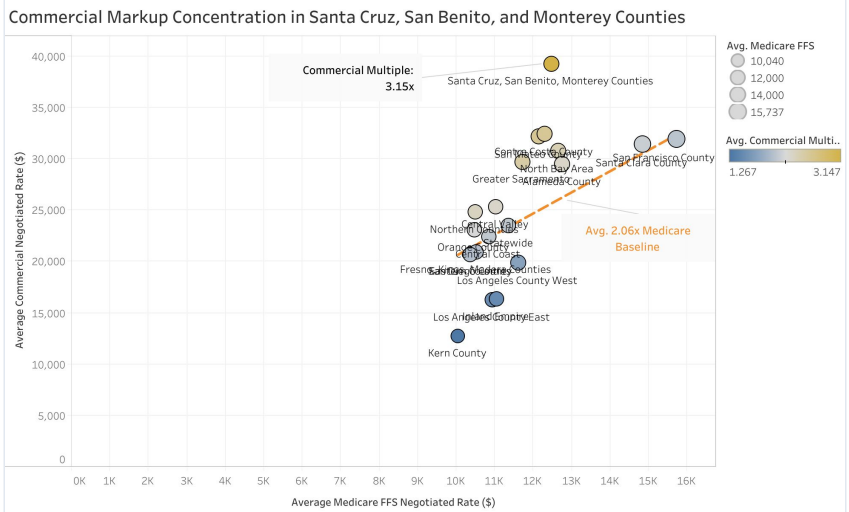
Implication I: Market Consolidation Mandates a Strategy Shift

So What?

High markups might be due to low hospital competition (Arbizu, 2024)(Bear, 2026).

What could this mean for CalPERS?

Traditional Methods of negotiation might not work.





BenitoLink
San Benito County's News

Donate

FACT BRIEFS

Fact Brief: Is San Benito's only hospital closing?

by **Jenny Mendolla Arbizu** January 9, 2026



Implication 2: Hidden Costs in Local Contracts Demand a Shift in Auditing

Insight:

A sizable gap within the same city means there may be other factors contributing, besides region costs.

What could this mean for CalPERS?

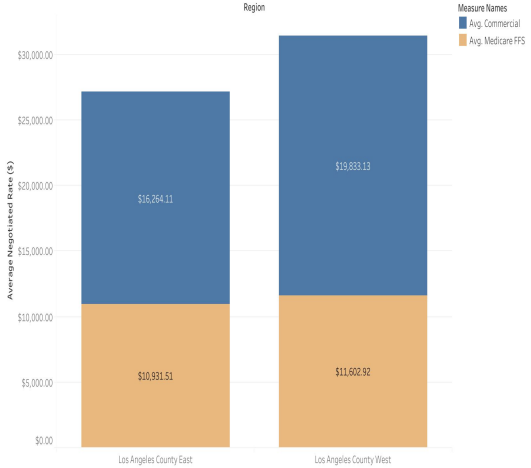
Only focusing on region may be inefficient.

East vs. West LA



VS







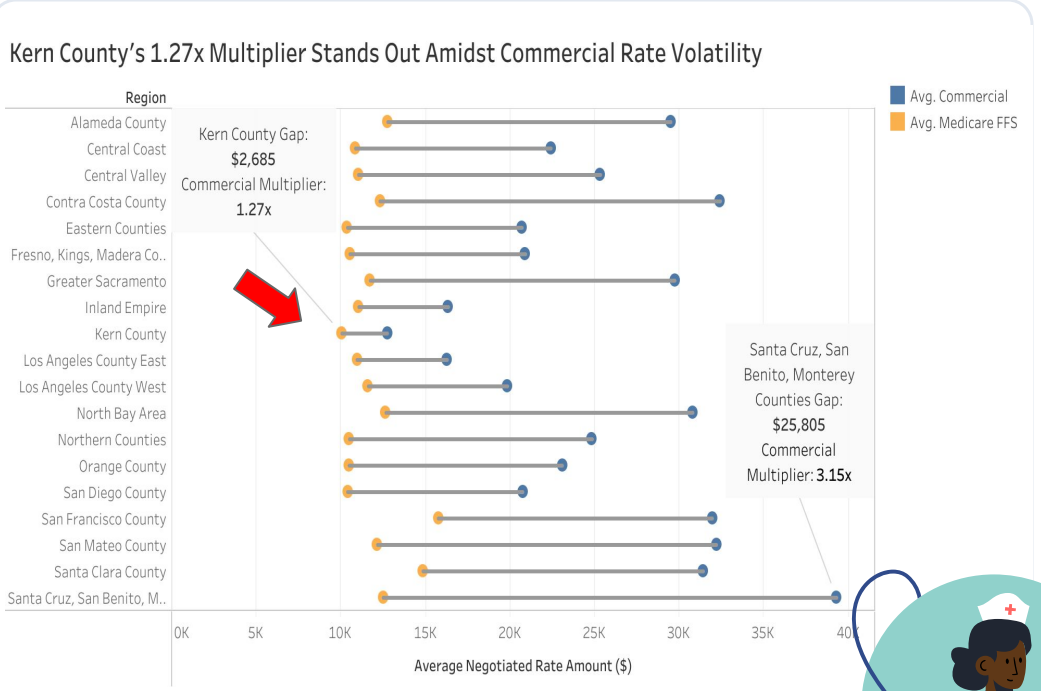
Implication 3: Kern County Proves it is Possible to Use Medicare FFS As A Baseline

Insight:

Kern County's 1.27x multiplier is the closest to matching Medicare FFS.

What could this mean for CalPERS?

CALPERS may be able to look into what Kern County is doing right, and use Medicare FFS as a baseline.





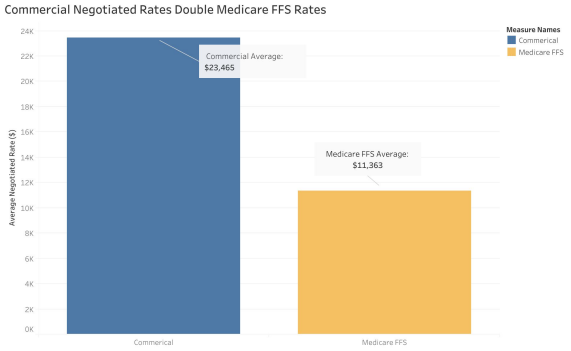
Implication 4: Possible Legal Vulnerability from Unjustified Markups

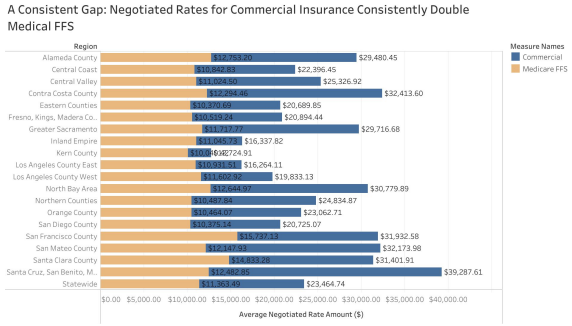
Insight:

Massive gaps between Medicare FFS and commercial prices might indicate price inflation.

What could this mean for CalPERS?

Calpers has the legal responsibility to ensure funds are spent solely on medical care.







OUR FOUR RECOMMENDATIONS

2.06x Commercial plans pay 2.06x what Medicare pays for comparable inpatient stays in California (severity-adjusted, HCAI 2023).

01. ANCHOR TO MEDICARE

Cap payments at a multiple of Medicare

02. COVER TRAVEL

Steer planned surgery to high-value hospitals

03. AUDIT HOSPITALS

Compare prices hospital by hospital

04. TRACK THE DATA

Benchmark and document with claims data

01 ANCHOR PRICES TO MEDICARE

RECOMMENDATION

- Pay hospitals a set multiple of Medicare, such as 200%
- Stop negotiating from hospitals' billed charges
- Start where markups are highest: Santa Cruz / San Benito / Monterey

Already working: Montana cut payments to 220–225% of Medicare. Kern County already pays just 1.27x.

IMPACT

3.15x → 2.0x

Santa Cruz / San Benito / Monterey markup with a 200% cap



$14,500

Saved per stay in that region



$81M

Saved by Oregon's 200% cap for state employees



02 COVER TRAVEL FOR SURGERY

RECOMMENDATION

- Contract with top hospitals at bundled rates for hip and knee replacements
- Waive cost-sharing; cover travel for the patient and a caregiver

WALMART'S RESULTS

15%
lower cost per case

70%
fewer readmissions

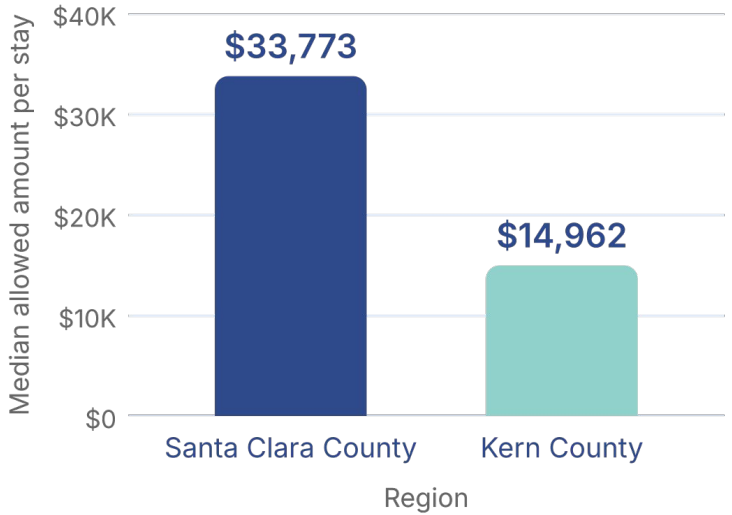
1.5 wks
faster return to work

IMPACT

The median commercial stay costs about $18,800 more in Santa Clara than in Kern, enough to cover travel.

Median Commercial Cost per Inpatient Stay

California, 2023 (HCAI, unadjusted)



03 AUDIT PRICES HOSPITAL BY HOSPITAL



RECOMMENDATION

- Compare each hospital's negotiated rates using price transparency files
- Tiered networks: lower copays at fairly priced hospitals

IMPACT

- Exposes overpricing that regional averages hide
- Pinpoints outlier hospitals to renegotiate and steers members to cheaper ones nearby

04 USE CLAIMS DATA FOR OVERSIGHT



01. GET THE DATA

Use CAA rights to get de-identified claims data; confirm no gag clauses



02. BENCHMARK YEARLY

Compare every contract to Medicare; document markups above a set threshold



03. TRACK THE TREND

Add HCAI 2018–2022 data to see if the 2.06x gap is growing

IMPACT

VERIFY

what CalPERS actually pays, not carrier summaries

DOCUMENT

a clear record of oversight for members

MEASURE

whether Recommendations 1–3 are working

Q INQUIRIES: OPEN QUESTIONS



01. WHY THE WIDE GAP?

Kern pays 1.27x Medicare; the Central Coast pays 3.15x. What drives it?



02. DOES TRAVEL PAY OFF?

Would high-cost regions save by covering travel for planned surgery?



03. IS THE GAP GROWING?

Is the 2.06x commercial-to-Medicare gap widening over time?

HOW WE COULD ANSWER THEM

MARKET STRUCTURE

Test consolidation, competition, and patient mix by county

COST MODEL

Weigh travel costs against the ~$18,800 Santa Clara vs. Kern gap

TREND DATA

Compare HCAI 2023 with 2018–2022 data

★ INSPIRATIONS: WHO PAVED THE WAY

RAND EMPLOYER STUDY



254%

Of Medicare | California: 300%+

WALMART COE PROGRAM



15%

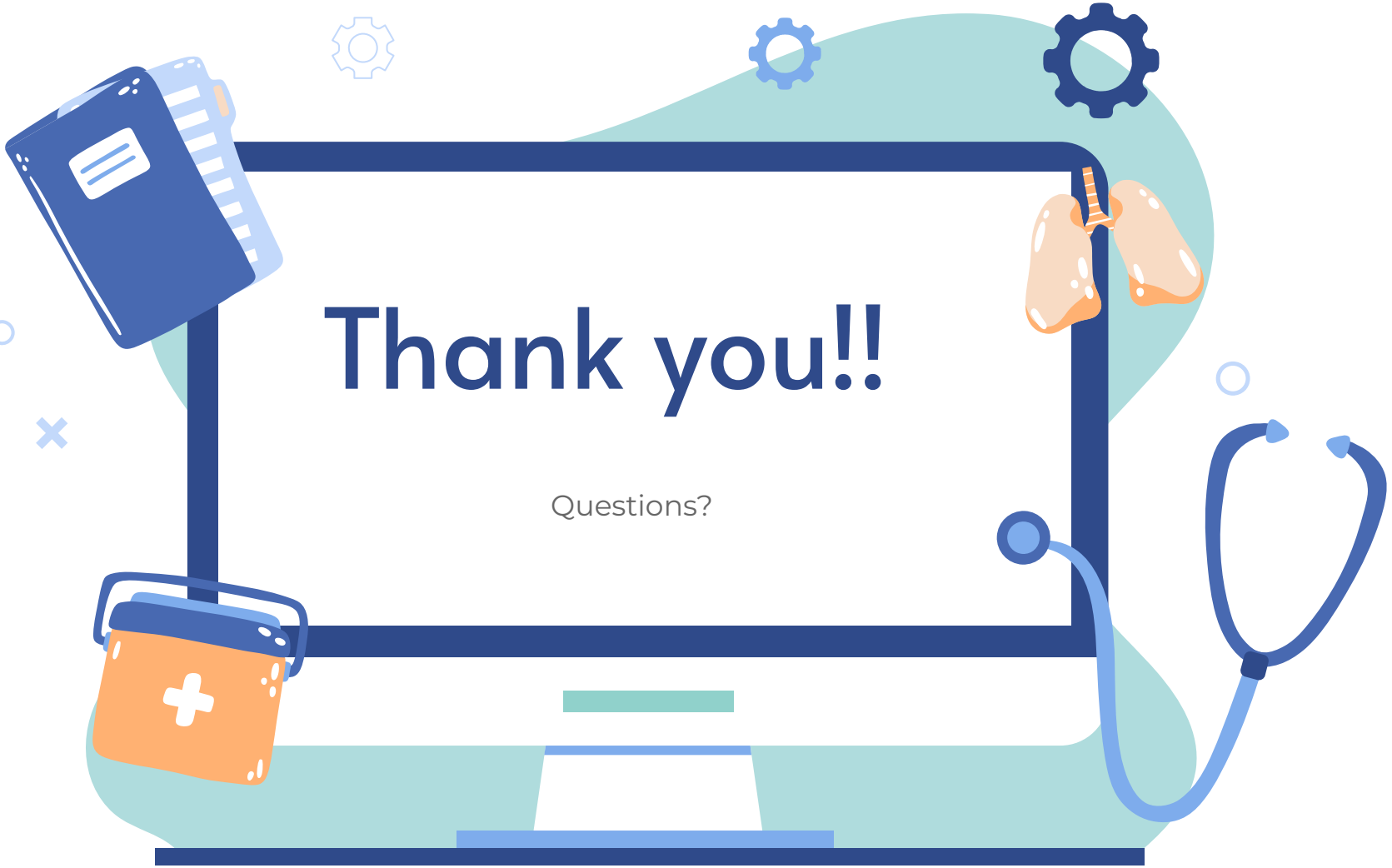
Lower cost per surgery

WHAT WE LEARNED

- Private plans pay 2.5x Medicare nationally; California tops 3x
- Sending patients to top hospitals cut readmissions 70%

HOW IT SHAPED OUR RECOMMENDATIONS

- RAND → Rec. 01: anchor hospital prices to Medicare
- Walmart → Rec. 02: cover travel for planned surgery


Thank you!!
Questions?

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